

TUITMR (Scar Repair Through a Scope)

WARWIKI

Patient Information · A repair of a scar at the bladder neck with no outside cut

This procedure repairs a tight scar at the bladder neck, or where the bladder was joined to the urethra after prostate surgery. It is done fully through a scope, with no outside cut. Your surgeon opens the scar, then stitches healthy bladder lining across it. The goal is for the area to heal open instead of scarring shut again.

About This Procedure

After prostate surgery or radiation, a tight scar can form at the **bladder neck** or at the join between the bladder and urethra. It can block the urine stream. Just cutting the scar often lets it close again, because it heals over raw scar.

TUITMR stands for transurethral incision with transverse mucosal realignment. It works in two steps:

- The surgeon **cuts the scar** through a scope to open the channel.
- With a special **stitching tool**, healthy bladder lining is pulled across and stitched over the cut.

Benefits and Risks in Numbers

Early results come from one center and follow men for about 1 to 2 years. In men with a scar after prostate surgery for enlargement:

8 in 10

needed no further procedure after one treatment

more than
9 in 10

had an open channel after up to two treatments

about
1 in 8

had a problem afterward, most often a minor urine infection

Numbers are averages from published studies. Your own risk may differ; ask your team.

What to Know

It is a newer technique, and it has not been compared directly with other repairs. In early, small studies, men did not develop new leakage. If you leaked before, or had radiation, leakage may be easier to notice once the channel is open.

Learn the Terms

Bladder neck

The outlet where the bladder funnels into the urethra.

Bladder neck contracture

A tight scar at the bladder neck that blocks the urine stream.

Anastomotic stenosis

A scar narrowing where the bladder was joined to the urethra after prostate removal.

Urethra

The tube that carries urine out of the body.

Mucosa (lining)

The healthy inner lining stitched across the scar to help it heal open.

Cystoscope

The thin camera the surgeon works through.

Incontinence

Leaking urine.

Catheter

A soft tube left in the urethra for a short time while it heals.

Will it hurt? You get anesthesia, so you feel nothing during the procedure. Afterward, expect mild burning and some pink urine for a few days. Most people go home the same day or after a short stay.

First days

Mild burning and pink urine. A catheter drains the bladder.

Catheter out

After a short time. Your team sets the date.

A few months

A camera check to make sure the area stays open.

After that

Tell your team if your stream slows again.

How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection. Any infection is treated first.
- Plan for a **catheter for a short time**, and arrange a ride home.

Tell your team ahead of time if you:

- Take a **blood thinner**
- Have had **radiation** or earlier treatments to this area
- Already leak urine

What Happens During the Procedure

- 1 You get anesthesia and antibiotics.
- 2 A scope is passed to the scar, and a thin wire is placed for safety.
- 3 The scar is stretched and cut to open the channel.
- 4 Healthy bladder lining is stitched across the cut, and a catheter is placed.

After the Procedure

- Drink extra water while your urine is pink.
- Ask your team when you can return to heavy lifting and straining.

Call the office that did your procedure if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Burning or pain that gets worse instead of better
- Leaking urine that is new or bothers you

Go to an emergency department if:

- You cannot urinate after the catheter is out and your bladder feels full or painful
- Heavy bleeding, or urine like red wine or with clots
- You cannot reach the office and are worried

Three Things to Remember

1. The scar is opened and covered with healthy lining so it can heal open.
2. It is a newer technique. Some men need a second treatment.
3. A follow-up camera check shows whether the area stays open.